



STAR RATINGS MOTION /// ONE-OFF STRIKE ROOM

Blue Shield of California

Buying-committee strike sequences for the cleanest cliff-edge book in the universe: every Medicare contract a half step from the bonus line, in the year the floors run leaner.

3 × 3.5★

EVERY MA
CONTRACT,
HALF A STEP
UNDER 4.0

**~\$33M /
yr**

FORGONE QBP,
ESTIMATE FROM
PUBLIC CMS
DATA

8 seats

COMMITTEE
SEQUENCED, 2
EXTENDED ARCS

4 days

STAGGERED
ENTRY, ONE
THESIS, EIGHT
ANGLES

Why Blue Shield, why now

Motion: Star Ratings cliff-edge (star_ratings) · **Product angle:** Queue Optimizer for member-facing operations, Back Office Optimizer for the processing work behind the service measures · **Target:** Blue Shield of California (California Physicians' Service) · **AE:** Jean-Anne McMahon · **Status:** send-ready draft, human approval gate not yet cleared; all 8 emails resolved via Clay and ZeroBounce-swept Jul 16: 8/8 valid (6 carry catch_all sub-status, the domain accepts all mail; Karrobi and Uhm verified clean)

The forcing function

The cleanest cliff-edge book in the universe colliding with a public cost mandate. All three of Blue Shield of California's Medicare Advantage contracts in the 2026 sub-4.0 universe sit at exactly 3.5 stars, one half step under the 4.0 bonus line, roughly \$33M a year in forgone Quality Bonus Payments by our estimate from public CMS data. No full-star climbs, no turnaround story required. At the same time the company is publicly shrinking the workforce that decides those measures: a third layoff round in twelve months takes effect July 28, 2026, under a new parent (Ascendium, Jan 2025) and a CEO seat that turned over twice in 2025. The floors are being asked to hold service with fewer people during the exact year the next rating is being set.

The one thesis every seat hears: The half star between Blue Shield and the bonus line is decided on the operations floor during peak weeks, and the capacity to hold it with a smaller roster already exists inside each day. Each seat hears it from their own chair. Blue Shield of California is a confirmed non-customer (not in the Jul 10 active-customer file; Blue Cross Blue Shield of North Carolina IS a customer and is a different company, never referenced).

Merge fields: {{sender}} first name, Touches 1-3 and voicemail intros · {{sender_full_name}} Touches 4-5.

Account snapshot

ITEM	VALUE	SOURCE
MA contracts below the 4.0 line	Three, all at 3.5 overall for 2026: H4937 (~45,600 members), H2819 (~15,000), H5928 (~7,900); ~68,500 members total	CMS 2026 Star Ratings release (Oct 8 2025) + Jun 2026 enrollment, canonical universe file
Forgone Quality Bonus Payments	~\$33M/year gross across the three contracts (estimate from public CMS data + KFF/MedPAC methodology)	Canonical universe file, 2026 cycle
Service-measure position	Customer-service domain stars in the mid-3s on all three contracts; the execution slice, not a clinical rebuild	CMS 2026 measure-level stars
Layoff round 3	69 positions effective Jul 28 2026, six CA counties; company: "we regularly adjust our staff based on business needs"	Becker's Payer, Jul 2026
Layoff round 2	69 positions effective Apr 8 2026 (Rancho Cordova, El Dorado Hills)	WARN filings via edhat, Apr 2026
Cumulative cuts	~300 positions eliminated since Jul 2025, plus hundreds in 2025 reporting	FQHC Talent Intel (Apr 2026), Becker's
Restructure	Ascendium formed as nonprofit parent Jan 2025 (Blue Shield of CA, Altais, Stellarus); Paul Markovich moved up to parent	Modern Healthcare, Jan 2025
CEO churn	Lois Quam named CEO Jan 2025, exited Mar 2025; CFO Mike Stuart interim, permanent CEO Aug 27 2025	Healthcare Dive, Aug 2025
Scale	~6M members, ~\$27B revenue, ~8,000 employees, HQ Oakland	Company statements, 2025-2026
Modernization posture	Contact Center Modernization program actively staffed (Principal PM hired Mar 2026)	LinkedIn, public profile
Customer status	Confirmed non-customer, eligible for new-logo motion	Active-customer file, Jul 10

Why now: the measurement year that sets the next rating is running right now and is more than half spent. Every peak week Blue Shield runs short-handed between now and year end is priced into the next October release before it publishes. The three

contracts are all fast money, a half step from the line, and the layoff rounds land directly on the operations that decide the service measures.

Internal targeting note (never in copy): the Elk Grove / Rancho Cordova / El Dorado Hills geography of the cuts overlaps the call-center operations footprint (Lambert, Mathews, Wynant all sit in greater Sacramento). The Contact Center Modernization program (Wynant, Mar 2026) means budget and executive attention are already on this floor; orchestration rides that program rather than competing with it. CMS's June 2026 recalculation of some 2026 ratings (litigation-driven) stays OUT of copy: it reads as "the measure is litigable," which undercuts the execution thesis, same lesson as the retired UHC \$190M figure.

Buying committee

#	NAME	TITLE	TIER	EMAIL STATUS	ANGLE
1	Lisa Lambert	Senior Director, Call Center Operations (in seat since 2014)	1 Champion	lisa.lambert@blueshieldca.com (ZB valid Jul 16, catch_all domain)	QO
2	Carolyn Mathews	Sr. Director Customer Service, Government Markets (May 2025)	1/5	carolyn.mathews@blueshieldca.com (Clay deep-find; ZB valid Jul 16, catch_all domain)	QO
3	George (Brad) Huber	Senior Director of Medicare Plan Performance (Sep 2025)	5	george.huber@blueshieldca.com (Clay deep-find; ZB valid Jul 16, catch_all domain)	QO + BOO
4	Donald Wynant	Principal, Program Manager, Contact Center Modernization (Mar 2026)	3	donald.wynant@blueshieldca.com (ZB valid Jul 16, catch_all domain)	QO
5	Alex Furman	Vice President, Medicare Performance (Nov 2024)	2	alex.furman@blueshieldca.com (ZB valid Jul 16, catch_all domain)	QO + BOO
6	Syng Karrobi	Senior Director IT, Digital/Marketing/Customer Care (2021)	3	syng.karrobi@blueshieldca.com (ZB valid Jul 16, verified clean)	QO
7	Alex Uhm	VP and General Manager of Medicare (Oct 2023)	2 Economic	alex.uhm@blueshieldca.com (ZB valid Jul 16, verified clean)	Platform
8	Christine McKinney	VP, Customer Experience and Digital Transformation (Nov 2024)	4 Exec	christine.mckinney@blueshieldca.com (ZB valid Jul 16, catch_all domain)	QO

LinkedIn: Lambert /in/lisa-lambert-b585104/ · Mathews /in/carolyn-mathews-4752b4163/ · Huber /in/george-brad-huber-97129865/ · Wynant /in/donaldwynant/ · Furman /in/alex-furman-965694161/ · Karrobi /in/syng-karrobi-8bb7824/ · Uhm /in/alexuhm/ · McKinney /in/christine-mckinney-2237b4/

Messaging level: company-level narrative for every seat except Huber, whose title (Medicare Plan Performance) clears him for the contract-specific view: the three contracts named, the ~\$33M estimate, and the one humility clause in the committee. Mathews leans Government Markets without contract claims. Nobody else gets an org-structure assumption.

Bench (second ring, fresh one-off pull on Nathan's go): Andy Stover (Senior Manager, Workforce Management and Analysis), Julianne Holloway (Executive Director,

Medicare Duals), Laetitia Barrad (Director, Clinical Quality, Medicare Stars; clinical side, weaker product fit).

What reps can and cannot say

Say: every figure in the snapshot table, always attributed ("by the public CMS release," "by public reporting on the July round"); the three contracts and ~\$33M only as an estimate from public CMS data; what Intradiem is; and, in replies only, the Humana story as Humana told it publicly (first-year in-year return, 7X ROI five years in, 2 hours of capacity per agent per month, public SWPP webinar).

Do not say: any Intradiem-computed savings, ROI, idle-percentage, or recovered-capacity number for Blue Shield; any customer outcome except Humana's; any claim about Blue Shield's org structure, internal tooling, or which layoffs hit which floor; the UHC \$190M court figure; the CMS June 2026 recalculation; any hard measure-level star in copy beyond "the service measures sitting in the mid-3s," labeled an estimate.

Star-level precision: all three contracts are at 3.5, a genuine half star under the line. There is no 3.0 block at this account; never manufacture one. "Three contracts" is count-verified against the canonical CMS file (Jul 16).

Cadence and committee entry

Five touches per contact across ~9 business days: **T1** Email 1 · **T2 (+1d)** LinkedIn connect · **T3 (+3d)** Voicemail + LinkedIn message · **T4 (+5d)** Email 2, new angle · **T5 (+8d)** Breakup. Every voicemail touch carries an if-they-answer live script; a pickup runs the script, never a read-out of the message. All 8 emails cleared the Jul 16 ZeroBounce sweep as valid. blueshieldca.com is a catch-all domain, so six addresses (including the deep-find pair, Mathews and Huber) carry catch_all sub-status: the server accepts all mail, so delivery is probable but not mailbox-guaranteed. The LinkedIn touch running parallel to every email is the built-in hedge; no sequence changes needed.

Extended arc, must-win seats only (Furman and Uhm): after Touch 4 the arc escalates before the breakup: **live call #1** (full script), **live call #2** (short), **pattern interrupt** (different format), then the breakup lands as the hail-mary. Escalation order is soft to direct to persistent, never inverted to hit a date.

Staggered entry: **Lambert and Huber Day 1, Mathews and Wynant Day 2, Furman and Karrobi Day 3, Uhm and McKinney Day 4.** Champions and the Stars operator open the account, executives enter after the floor has had time to mention the outreach internally. Eight sequences carry eight different openers and eight different CTAs, so side-by-side forwards inside Blue Shield read as one thesis from eight chairs, not a blast. Subjects never repeat across contacts; send hours stagger within each day. If an email is at deliverability risk, the LinkedIn touch carries the opener.

Lisa Lambert

Senior Director, Call Center Operations

TOUCH 1 (DAY 1) • EMAIL

Subject: the half star your floor decides

Lisa, twelve years running Blue Shield's call centers means you've seen what a rating year does to a floor. This one is sharper than most. All three Medicare contracts sit at 3.5 stars, a half step from the 4.0 bonus line, and by public reporting the operation is holding that line with fewer people after three rounds of cuts in a year. The half star isn't won in a scorecard review. It's won in the weeks volume runs hardest, when service either holds or slips while the measures are watching. Intradiem orchestrates the workforce off real-time conditions so it holds. Worth twenty minutes on where a floor your size typically loses it?

{{sender}}

TOUCH 2 (DAY 2) • LINKEDIN CONNECTION REQUEST

Lisa, I work the operations side of health plan Stars, the gap between the scorecard and what happens on the floor when volume spikes and rosters shrink. Given how long you've run Blue Shield's centers, would value connecting.

TOUCH 3 (DAY 4) • VOICEMAIL

Lisa, this is {{sender}} with Intradiem. I sent you a note on the half star between Blue Shield's Medicare contracts and the bonus line, and why it gets decided on your floor during peak weeks, especially in a year of holding service with fewer people. I can show where floors your size usually lose it. I'm at [number], or reply to the email. Thanks Lisa.

TOUCH 3 (DAY 4) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Lisa, this is {{sender}} with Intradiem. I sent you a note this week on the Medicare book. Can I take thirty seconds live?

THE THIRTY SECONDS

All three Medicare contracts sit at 3.5, a half step from the bonus line, and the measures that decide whether they clear are service measures, set on your floor during peak weeks. In a year of running leaner, the only capacity that holds that line without a req is the idle minutes already inside each day, acted on in real time. That's the whole note.

CLOSE

Worth twenty minutes on where floors your size usually lose the half star?

TOUCH 3 (DAY 4) • LINKEDIN MESSAGE

Lisa, left you a voicemail too. Short version: all three Medicare contracts are a half step from the bonus line, and whether they clear is an execution question during peak weeks on your floor. Twenty minutes on where that gap usually opens?

TOUCH 4 (DAY 6) • EMAIL

Subject: what gets cut before it shows

Hi Lisa, the pattern every lean year repeats on a service floor: coaching and off-phone time go first, service holds for a quarter on effort, then the measures move after it's too late to act inside the window. Recovering the idle minutes already inside each day flips that, the same capacity funds the coaching and absorbs the spikes without a req. That's what Intradiem does in real time, on top of the WFM you already run. Twenty minutes and I'll walk how floors your size fund the peak weeks from inside the schedule.

{{sender_full_name}}

TOUCH 5 (DAY 9) • EMAIL

Subject: leaving it here

Hi Lisa, I'll stop here. One thought worth keeping: when a floor holds through a rating year nobody sees it in October, and when it slips everybody does. The difference is decided by what the roster does with its minutes during the peak weeks between now and year end, not by anything that happens after the release. When that lands on your desk, the workforce-execution angle is where I'd start. Glad to help whenever it's useful.

{{sender_full_name}}

Carolyn Mathews

Sr. Director Customer Service,
Government Markets

EMAIL VIA CLAY DEEP-FIND, ZB GATE BEFORE SEND

TOUCH 1 (DAY 2) · EMAIL

Subject: the measures your phones answer for

Carolyn, taking over customer service for Government Markets a year before a cliff-edge rating cycle is a specific kind of inheritance. All three of Blue Shield's Medicare contracts sit at 3.5 stars for 2026, a half step from the 4.0 bonus line, and the fastest measures still moving this late in the window are the service measures your teams answer for every day: how fast members reach a person, what happens under load, whether follow-through holds in peak weeks. Intradiem acts on real-time floor conditions so those measures hold when volume doesn't cooperate. Worth twenty minutes on which service measures still move before the window closes?

{{sender}}

TOUCH 2 (DAY 3) · LINKEDIN CONNECTION REQUEST

Carolyn, sent you a note on the service measures riding on Government Markets phones this rating year, with the whole Medicare book a half step from the bonus line. Would value connecting here too.

TOUCH 3 (DAY 5) · VOICEMAIL

Carolyn, this is {{sender}} with Intradiem. I reached out about the service measures on the Medicare book, the ones your teams answer for daily, with all three contracts a half step from the bonus line. The window to move them is this year's peak weeks, not October. Twenty minutes on which ones still move? I'm at [number]. Thanks Carolyn.

TOUCH 3 (DAY 5) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Carolyn, this is {{sender}} with Intradiem. I sent you a note on the Medicare service measures. Thirty seconds live, and you tell me if it's worth more?

THE THIRTY SECONDS

The Medicare contracts all sit a half step under the bonus line, and this late in the measurement year the levers still moving are the service measures, decided on your phones during peak weeks. Intradiem acts on real-time conditions so service holds under load, on top of the systems you already run. New seat, cliff-edge book, that's why I called you specifically.

CLOSE

Worth twenty minutes on which service measures still move before the window closes?

TOUCH 3 (DAY 5) • LINKEDIN MESSAGE

Carolyn, left you a voicemail as well. Short version: this late in the measurement year, the levers still moving on the Medicare book are the service measures your phones own, and holding them under load is an execution question. Twenty minutes on which ones still move?

TOUCH 4 (DAY 7) • EMAIL

Subject: a first rating year sets the baseline

Hi Carolyn, one thing about timing that seat inherits: the rating published next October is being set by what your phones do between now and December, and the year after that gets judged against whatever this year produces. Holding the service measures now is cheaper than explaining them later, and the capacity to hold them already sits inside each day as idle minutes nothing currently acts on. That's the layer Intradiem runs in real time. Twenty minutes and I'll walk where government-markets service operations usually find it.

{{sender_full_name}}

Subject: stopping here

Hi Carolyn, I'll stop here. One parting thought: every service measure that slips this year gets defended in a meeting next year, and every one that holds never comes up again. The difference is what the floor does during the peak weeks that are already on your calendar. When that becomes the conversation, this seam is where I spend my time. Glad to help whenever the timing works.

{{sender_full_name}}

George (Brad) Huber

Senior Director of
Medicare Plan
Performance

EMAIL VIA CLAY DEEP-FIND, ZB GATE BEFORE SEND

THE ONE CONTRACT-SPECIFIC SEAT

TOUCH 1 (DAY 1) • EMAIL

Subject: three contracts, one half step

Brad, since you own Medicare plan performance I'll be specific. By our estimate from public CMS data, Blue Shield's three MA contracts, H4937 with about 45,600 members plus the two smaller books, all sit at 3.5 for 2026, which leaves roughly \$33M a year in Quality Bonus Payments on the table with no full-star climb required anywhere. You'll know the exact picture better than I will. On contracts this close to the line, the gap is largely execution: whether service and the processing work behind it hold during peak weeks. Intradiem orchestrates the contact center and back office off real-time conditions so they do. Twenty minutes on the three contracts and which measures still move them this cycle?

{{sender}}

TOUCH 2 (DAY 2) • LINKEDIN CONNECTION REQUEST

Brad, you took the plan-performance seat right as the October release put all three contracts at 3.5. Reached out on which measures still move them this cycle; would value connecting here too.

TOUCH 3 (DAY 4) • VOICEMAIL

Brad, this is {{sender}} with Intradiem. I reached out about the three contracts sitting at 3.5, all a half step from the bonus line, roughly \$33M a year by public CMS math, and how much of that gap is execution holding during peak weeks. Twenty minutes on which measures still move this cycle? I'm at [number]. Thanks Brad.

TOUCH 3 (DAY 4) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Brad, this is {{sender}} with Intradiem. I sent you a note on the three contracts at 3.5. Can I take thirty seconds?

THE THIRTY SECONDS

All three contracts sit a half step from the bonus line, roughly \$33M a year by public CMS math, and there's no full-star climb anywhere on the book, which makes it the cheapest kind of Stars gap to close. This late in the window the movers are the service and admin measures, and those are decided by execution during peak weeks. That's the piece Intradiem runs in real time.

CLOSE

Twenty minutes on the three contracts and which measures still move them this cycle?

TOUCH 3 (DAY 4) • LINKEDIN MESSAGE

Brad, left you a voicemail too. Short version: three contracts, all a half step from the line, no full-star climbs, and the measures still moving this cycle are execution measures. Twenty minutes on which ones still swing?

TOUCH 4 (DAY 6) • EMAIL

Subject: the next rating is being set now

Hi Brad, one point on timing for the book specifically. The measurement that sets the next rating is running across this year and is already more than half spent, so the three contracts are being decided right now, in the peak weeks between now and year end, not on the October release. Every week execution holds is a week toward clearing the line; every week it slips is priced in before the rating ever publishes. Intradiem makes that execution consistent under load, contact center and the back-office work behind it. Twenty minutes and I'll walk the plan-performance view of where the half star is usually won or lost.

{{sender_full_name}}

Subject: last note on the three contracts

Hi Brad, I'll stop for now. One parting thought: a book where every contract sits at 3.5 is the cheapest half star in Medicare Advantage to buy back, and that shape is rare and temporary, one bad measurement year splits it into full-star climbs. The window to move it is the peak weeks happening now. When protecting it becomes the priority, the workforce-execution angle is where I'd start. Glad to help whenever the timing is right.

{{sender_full_name}}

Donald Wynant

Principal Program Manager, Contact Center Modernization

TOUCH 1 (DAY 2) • EMAIL

Subject: the layer most modernizations bolt on late

Don, contact center modernization programs usually get the platforms right and leave one layer for later: the thing that acts inside the day, in real time, when conditions shift faster than the plan. Routing, WFM, and analytics plan the day and report on it afterward; nothing in the standard blueprint acts on the minutes in between. At Blue Shield that layer has unusual stakes, because the Medicare book sits a half step from the bonus line and the measures that move it are decided on the floors your program is redesigning. Intradiem is that layer, an overlay on whatever stack the program lands on, not another migration. Worth twenty minutes on where it slots into the roadmap before the blueprint hardens?

{{sender}}

TOUCH 2 (DAY 3) • LINKEDIN CONNECTION REQUEST

Don, I work the real-time orchestration layer of contact center stacks, the piece that acts inside the day on top of the WFM and routing a modernization program puts in. Given what you're building at Blue Shield, seemed worth connecting.

TOUCH 3 (DAY 5) • VOICEMAIL

Don, this is {{sender}} with Intradiem. I sent you a note on the layer modernization programs usually bolt on late, the one that acts on floor conditions in real time, riding on the platforms you're already putting in rather than competing with them. Cheaper to design in than retrofit. I'm at [number], or reply to the email. Thanks Don.

TOUCH 3 (DAY 5) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Don, this is {{sender}} with Intradiem. I sent you a note on the modernization program. Got thirty seconds?

THE THIRTY SECONDS

Everything in a modernization blueprint plans the day, routes the work, or reports on it after. The layer that acts inside the day, in the minutes between scheduled work, usually gets bolted on two years later at retrofit prices. It's an overlay on the WFM and routing you're standing up, and at Blue Shield it carries the Medicare service measures with it.

CLOSE

Worth twenty minutes on where it slots into the roadmap while the blueprint is still soft?

TOUCH 3 (DAY 5) • LINKEDIN MESSAGE

Don, left you a voicemail too. Short version: the real-time action layer is the piece modernization blueprints defer, and deferring it costs retrofit prices plus every peak week in between. Twenty minutes on where it fits in yours?

TOUCH 4 (DAY 7) • EMAIL

Subject: what the program gets judged on

Hi Don, one framing from the buyer's side of these programs: a modernization is judged less on the platforms it ships than on what the floor does differently at 2pm on a Tuesday when volume spikes. That difference is real-time orchestration, and it's the piece that makes the rest of the stack visible to the business, because it turns plans into action while the conditions still hold. It rides on what you're building, no rip-and-replace, and it's cheapest to scope now. Twenty minutes and I'll walk the integration view against a modernization architecture.

{{sender_full_name}}

TOUCH 5 (DAY 10) • EMAIL

Subject: closing the loop

Hi Don, I'll stop here. One parting thought: platforms get replaced every few years, but the unacted minutes inside the day survive every migration, because no platform in the standard blueprint owns them. Programs that claim that layer early are the ones the floor still thanks two years later. When it's worth a look, that's the piece I work. Glad to help whenever the timing fits.

{{sender_full_name}}

Alex Furman

Vice President, Medicare
Performance

EXTENDED ARC (MUST-WIN)

TOUCH 1 (DAY 3) · EMAIL

Subject: where the next rating is being set

Alex, the measurement year that sets Blue Shield's next Medicare rating is more than half spent, and the whole book is sitting a half step from the 4.0 line. This late in a window, the levers that still move are not the clinical programs that needed to start in January. They're the service and admin measures, decided week by week by whether operations hold under load, and by public reporting the floors are holding it with fewer people this year. Intradiem orchestrates the contact center and back office off real-time conditions so execution doesn't decide the rating by default. Worth twenty minutes on what still moves this cycle and what holding it returns?

{{sender}}

TOUCH 2 (DAY 4) · LINKEDIN CONNECTION REQUEST

Alex, I work the execution side of Medicare Stars, the measures that are still movable in the back half of a measurement year. With Blue Shield's book a half step from the line, would value connecting.

TOUCH 3 (DAY 6) · VOICEMAIL

Alex, this is {{sender}} with Intradiem. I sent you a note on the back half of the measurement year, where the movable measures are the operational ones, and Blue Shield's whole book sits a half step from the bonus line. I can walk what still moves this cycle and what holding it returns. I'm at [number], or reply to the email. Thanks Alex.

TOUCH 3 (DAY 6) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Alex, this is {{sender}} with Intradiem. I sent you a note on the measurement year. Thirty seconds, and you tell me if it's worth more?

THE THIRTY SECONDS

The year that sets the next rating is more than half gone, and every contract on the book is a half step from the line. The measures still movable this late are the operational ones, service levels, responsiveness, the processing behind them, and they're decided by execution under load during peak weeks. That's the layer Intradiem runs, in real time, on top of the WFM already in place.

CLOSE

Worth twenty minutes on what still moves this cycle and what holding it returns?

TOUCH 3 (DAY 6) • LINKEDIN MESSAGE

Alex, left you a voicemail too. Short version: in the back half of a measurement year the movable Stars measures are operational, and the book is a half step from the line. Twenty minutes on what still moves and what it returns?

TOUCH 4 (DAY 8) • EMAIL

Subject: execution is the cheapest measure category

Hi Alex, one performance framing worth having. Moving a clinical measure means changing member and provider behavior over quarters. Moving a service measure means changing what the operation does under load next week, with people already on the payroll. Same half star, very different cost and very different clock, and the service side is the one still open this cycle. That's the slice Intradiem exists for: real-time orchestration that keeps service and processing steady when volume runs hardest. Twenty minutes and I'll walk the performance view of which measures pay back fastest from the floor.

{{sender_full_name}}

TOUCH 5 (DAY 10) · LIVE CALL #1, FULL SCRIPT

OPENER (ASK PERMISSION, THEN STOP TALKING)

Alex, this is {{sender}} with Intradiem. Cold call, I'll own that. Thirty seconds, and you decide if it's worth more?

THE THIRTY SECONDS

Every contract on Blue Shield's Medicare book is a half step from the bonus line, roughly \$33M a year by public CMS math, and the measurement year that decides the next rating is more than half spent. The measures still open are the operational ones, and they're being set during peak weeks by floors running leaner than last year. Intradiem acts on real-time conditions so service holds under load. I'm asking for twenty minutes on which measures still move and what holding them returns.

IF "WE HAVE A STARS PROGRAM"

Every plan does, and it usually owns the clinical and pharmacy measures. The gap is the operational slice, where execution during peak weeks sets the number. This rides under the program you have, it doesn't compete with it.

IF "TALK TO MY STARS TEAM"

Happy to, and I will. The reason I called you first is that the fix is operational capacity, which crosses the performance and operations line, and that trade-off is made at your level.

IF "SEND ME SOMETHING"

Will do, and the one thing a deck can't answer is which of the three contracts' measures still swing this cycle. That's the twenty-minute part.

CLOSE

Does Thursday work, or is early next week better?

TOUCH 6 (DAY 12) · LIVE CALL #2, SHORT SCRIPT

Alex, {{sender}} at Intradiem again, I'll be quick. You've had two notes and a voicemail on one idea: the book is a half step from the line, the window is closing weekly, and the measures still open are decided by floor execution. Twenty minutes shows what still moves and what it returns. If it's a no, tell me no and I'll close the file. Fair enough?

TOUCH 7 (DAY 13) · PATTERN INTERRUPT: THREE-LINE EMAIL

Subject: three numbers

Three contracts at 3.5. Roughly \$33M a year on the table by public CMS math. Half a measurement year left to move it.

That's the whole argument. Twenty minutes tests whether it holds at Blue Shield.

{{sender_full_name}}

Subject: before I close the file

Hi Alex, I'll stop here. One parting thought: the performance seat gets judged on the October release, but the release is just the receipt. The purchase happens in the peak weeks between now and December, on floors that are running leaner than the ones that set last year's baseline. When the operational slice of the book becomes the priority, that's the seam I work. Glad to help whenever the timing is right.

{{sender_full_name}}

Syng Karrobi

Senior Director IT, Digital/Marketing/Customer Care

TOUCH 1 (DAY 3) • EMAIL

Subject: what the stack watches but doesn't do

Syng, the customer care stack you run plans the day, routes the work, and reports on what happened. What none of it does is act inside the day, in the minutes between scheduled work, when conditions shift faster than the plan. That gap matters more at Blue Shield than most places right now, because the Medicare service measures are a half step from the bonus line and the floors are running leaner this year. There's a workforce layer that closes it: real-time orchestration riding on the WFM, routing, and telephony you already own. No rip-and-replace. Worth twenty minutes on how it plugs into your stack?

{{sender}}

TOUCH 2 (DAY 4) • LINKEDIN CONNECTION REQUEST

Syng, sent you a note on the layer customer-care stacks watch but don't act on, the real-time piece between WFM and the floor. Given you own that stack at Blue Shield, seemed worth connecting.

TOUCH 3 (DAY 6) • VOICEMAIL

Syng, this is {{sender}} with Intradiem. I sent you a note on the layer that acts inside the day, on the minutes between scheduled work, riding on the care stack you already run rather than replacing any of it. I'm at [number], or reply to the email. Thanks Syng.

TOUCH 3 (DAY 6) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Syng, this is {{sender}} with Intradiem. I sent you a note on the customer care stack. Got thirty seconds?

THE THIRTY SECONDS

Everything in the stack plans the day, routes the work, or reports on it after. Nothing acts inside the day, in the minutes between scheduled work, when conditions shift faster than the plan. There's a workforce layer that does exactly that, riding on the WFM and telephony you already own. Overlay, not a migration, and at Blue Shield it carries Medicare service measures with it.

CLOSE

Worth twenty minutes on the integration view with your stack?

TOUCH 3 (DAY 6) • LINKEDIN MESSAGE

Syng, left you a voicemail too. Short version: the stack plans and reports, nothing acts in the gaps in real time, and that's the layer I work, on top of what you already own. Twenty minutes on the integration view?

TOUCH 4 (DAY 8) • EMAIL

Subject: the workforce half of digital

Hi Syng, one framing from the IT side of health plans. The digital investments carry the self-service half of member experience, and every call they deflect makes the remaining calls harder, longer, and more measured. The workforce half is what happens to those humans in real time, and it runs on the same logic as the digital half: signal comes in, action goes out, no human dispatcher in between. That's Intradiem, an overlay on the platforms you already run, standard integrations. Twenty minutes and I'll walk the architecture view against your stack.

{{sender_full_name}}

TOUCH 5 (DAY 11) • EMAIL

Subject: one last thought

Hi Syng, I'll stop here. One parting thought: every platform in the care stack will be replaced eventually, and the minutes between scheduled work will still be there afterward, unowned, because plans and reports don't act. The cheapest capacity in the operation lives in that seam. When it becomes worth a look, that's where I spend my time.

{{sender_full_name}}

Alex Uhm

VP and General Manager of
Medicare

TOUCH 1 (DAY 4) · EMAIL

Subject: the margin line hiding in the stars

Alex, running the Medicare P&L at a nonprofit that is publicly tightening its cost base, you already know the rarest kind of line item: one that moves without a benefit cut, a rate win, or a headcount add. By our estimate from public CMS data, Blue Shield's three MA contracts leave roughly \$33M a year in Quality Bonus Payments on the table, and all three sit at 3.5, a half step from the line, no turnaround required. The measures still open this cycle are operational, decided on the service floors during peak weeks. Intradiem holds them steady in real time with the roster you already pay for. Worth twenty minutes on the economics of the half point on your book?

{{sender}}

TOUCH 2 (DAY 5) · LINKEDIN CONNECTION REQUEST

Alex, I work the operations side of Medicare Stars economics, the execution slice between a 3.5 book and the bonus line. Given you run the Medicare business at Blue Shield, would value connecting.

TOUCH 3 (DAY 7) · VOICEMAIL

Alex, this is {{sender}} with Intradiem. I sent you a note on the bonus dollars sitting a half step away on the Medicare book, roughly \$33M a year by public CMS math, and why the measures still open this cycle are operational ones. Twenty minutes on the economics of the half point? I'm at [number], or reply to the email. Thanks Alex.

TOUCH 3 (DAY 7) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Alex, this is {{sender}} with Intradiem. I sent you a note on the Medicare book's bonus math. Can I take thirty seconds live?

THE THIRTY SECONDS

Three contracts, all at 3.5, roughly \$33M a year in bonus payments a half step away by public CMS math. No full-star climbs, so it's the cheapest kind of Stars gap in the industry to close. The measures still open this cycle are operational, and they're being decided by floors running leaner than last year. Intradiem holds service steady in real time with the roster you already pay for.

CLOSE

Worth twenty minutes on the economics of the half point on your book?

TOUCH 3 (DAY 7) • LINKEDIN MESSAGE

Alex, left you a voicemail too. Short version: roughly \$33M a year sits a half step away on the book by public CMS math, and the path to it this cycle is operational, not clinical. Twenty minutes on the economics?

TOUCH 4 (DAY 9) • EMAIL

Subject: what the bonus actually buys

Hi Alex, one step past the bonus math. QBP dollars fund the benefits that win enrollment, so a book that clears the line compounds: richer benefits, stronger AEP, more members earning the bonus. A book that misses runs that loop in reverse while competitors fund benefits Blue Shield has to match out of margin. The entry ticket is operational execution during this year's remaining peak weeks, using capacity already inside the day. That's the layer Intradiem runs. Twenty minutes and I'll walk what the half point returns against what holding it costs.

{{sender_full_name}}

TOUCH 5 (DAY 11) · LIVE CALL #1, FULL SCRIPT

OPENER (ASK PERMISSION, THEN STOP TALKING)

Alex, this is {{sender}} with Intradiem. I know I'm calling cold. Thirty seconds, and you tell me if it's worth more?

THE THIRTY SECONDS

You run a Medicare book where all three contracts sit at 3.5, which by public CMS math leaves roughly \$33M a year in bonus payments a half step away. The measures still open this cycle are operational, and by public reporting the floors are holding service with fewer people this year. The capacity that closes that gap is the idle minutes already inside each day, orchestrated in real time. No benefit cut, no rate dependency, no headcount add. I'm asking for twenty minutes on the economics of the half point.

IF "STARS IS MY PERFORMANCE TEAM'S LANE"

The measures are, and the trade-off isn't. This is operational capacity on the service floors, and whether that investment gets made is a general-manager call, which is why I called you.

IF "BUDGET'S TIGHT, WE'RE CUTTING"

That's the setting that makes this the right year. The bonus line is the one number that moves without adding heads, and missing it in a lean year costs more than holding it.

IF "SEND ME SOMETHING"

Will do, and the one thing a deck can't answer is what the half point is worth on your specific three contracts against this year's remaining window. That's the twenty minutes.

CLOSE

Does Thursday work, or is early next week better?

TOUCH 6 (DAY 13) · LIVE CALL #2, SHORT SCRIPT

Alex, {{sender}} at Intradiem again, quick one. You've had two notes and a voicemail on the same idea: roughly \$33M a year sits a half step away on your book, the window is closing weekly, and the path is operational. Twenty minutes on the economics. If it's a no, say no and I'm gone. Fair?

TOUCH 7 (DAY 14) · PATTERN INTERRUPT: LINKEDIN VOICE NOTE (WITH TEXT TL;DL)

Voice note (~25s): Alex, {{sender}} with Intradiem. One thought, twenty seconds. Three contracts at three and a half stars. Roughly thirty-three million a year, a half step away, by public math. And the measures that close it are set on the floor, this year, with the people already on payroll. Twenty minutes whenever it's worth it.

Text TL;DL (same message): Three contracts at 3.5, roughly \$33M a year a half step away by public CMS math, closed on the floor with the roster you already pay for. Twenty minutes whenever it's worth it.

TOUCH 8 (DAY 16) · BREAKUP EMAIL (HAIL-MARY)

Subject: final note

Hi Alex, I'll stop here. One parting thought: most of the levers on a Medicare P&L trade something for something, benefits for margin, growth for risk. The bonus line on a 3.5 book is the exception, it's bought with execution, and the purchase window is the peak weeks between now and December. When it becomes a priority, the workforce-execution angle is where I'd start. Glad to help whenever the timing is right.

{{sender_full_name}}

Christine McKinney

VP, Customer Experience and Digital Transformation

TOUCH 1 (DAY 4) • EMAIL

Subject: the half of experience that isn't digital

Christine, digital transformation carries the self-service half of member experience. The other half stays stubbornly human: how long a member waits, whether the answer is right the first time, what happens under load in a peak week. On the Medicare book that half is graded, the experience and service measures sit a half step from the 4.0 bonus line on all three contracts, and by public reporting the floors are holding it with fewer people this year. Intradiem keeps the human half steady by acting on floor conditions in real time. Worth twenty minutes on where experience erodes first when rosters tighten, and what holding it returns?

{{sender}}

TOUCH 2 (DAY 5) • LINKEDIN CONNECTION REQUEST

Christine, sent you a note on the non-digital half of member experience, the part that gets decided on the floor under load and graded by CMS. Worth connecting here too.

TOUCH 3 (DAY 7) • VOICEMAIL

Christine, this is {{sender}} with Intradiem. I sent you a note on the human half of member experience, the part that erodes quietly when volume outruns staffing and shows up in the Medicare measures two quarters later. There's a real-time layer that holds it. I'm at [number], or reply to the email. Thanks Christine.

TOUCH 3 (DAY 7) • IF THEY ANSWER (LIVE SCRIPT)

OPENER

Christine, this is {{sender}} with Intradiem. I sent you a note on the member experience measures. Thirty seconds live?

THE THIRTY SECONDS

Digital carries the self-service half of experience. The human half, wait times, first-contact resolution, follow-through under load, is graded on the Medicare book, and it erodes quietly when floors run lean, then shows up in the measures after the window to fix it has closed. Intradiem acts on floor conditions in real time so it holds, on top of the transformation work already underway.

CLOSE

Worth twenty minutes on where experience usually erodes first when rosters tighten?

TOUCH 3 (DAY 7) • LINKEDIN MESSAGE

Christine, left you a voicemail too. Short version: the digital half of experience is designed, the human half is defended, and this year it's being defended with fewer people while the measures watch. Twenty minutes on where it erodes first?

TOUCH 4 (DAY 9) • EMAIL

Subject: members don't file complaints, they answer surveys

Hi Christine, the uncomfortable mechanics of experience in a rating year: a member who waits too long mostly doesn't complain to Blue Shield, they answer a CMS survey months later, and by then the erosion is priced into the rating with no appeal on the operational side. The cheap fix is upstream and real-time, holding service in the moment volume spikes, funding coaching from minutes already inside the day. That's the layer Intradiem runs, alongside the digital work rather than instead of it. Twenty minutes and I'll walk how the experience measures connect to floor execution.

{{sender_full_name}}

TOUCH 5 (DAY 12) • EMAIL

Subject: last note

Hi Christine, I'll leave it here. One parting thought: experience lost in a lean year is bought back at a premium later, in the rating, in the remediation, and in the story told about both. The plans that avoid that buy-back are the ones whose floors hold in the moment, not the ones with the best post-mortem. When that becomes the conversation in your seat, this is the seam I work. Glad to help whenever the timing fits.

{{sender_full_name}}

Objection quick-handles

"We'll see where we land in October." October is a receipt for a purchase that already happened. The rating that publishes then was set by this year's peak weeks, and the one after it is being set right now. Waiting for the release means starting the next climb from whatever this year's execution produces. Twenty minutes now is the cheap version of that conversation.

"We have a Stars program." Every plan does, and it usually owns the clinical and pharmacy measures, the ones that need quarters of member and provider behavior change. The operational slice, service holding under load during peak weeks, is the piece that still moves this late in a window, and it rides under the program rather than competing with it.

"We have WFM." Keep it. WFM plans the day; nothing acts inside it, in the gaps between scheduled work, when the queue spikes and the plan is already stale. This is the layer that acts in those gaps, on top of the WFM you just named, and it covers the back-office work behind the service measures too.

"No budget, we're cutting." That's the year this matters most. The bonus line is the one number on the Medicare book that moves without adding heads, and the capacity that moves it is already on the payroll as idle minutes. If it can't justify itself against the half star, twenty minutes proves that too.

"Send me information." Will do. The one thing a deck can't answer is which of the three contracts' measures still swing this cycle and where your floors lose the half star today; that's a twenty-minute conversation. I'll send the overview and we can find time if it's worth it. *Optional verified proof (1:1 replies only, labeled public):* Humana, on the record: first-year in-year return, 7X ROI five years in, 2 hours of capacity per agent per month (public SWPP/Intradiem webinar).

"We're evaluating other tools / we have Verint or NICE." Good, keep them, they plan and record. The layer that acts on real-time conditions between the plan and the floor is the piece that decides whether the plan survives a peak week. This is that layer, and it rides on what you already run.

Claims basis and execution notes

Claims basis.

Every Stars figure is an estimate from the public CMS 2026 Star Ratings release (Oct 8 2025), June 2026 enrollment, and KFF/MedPAC bonus methodology, labeled as such in copy; "three contracts" is count-verified against the canonical CMS universe file (Jul 16 2026). The half-star framing is precise: all three contracts sit at 3.5 overall; no 3.0 block exists at this account and none is claimed. Cost and restructuring facts (layoff rounds and effective dates, Ascendium, CEO timeline) are public reporting (Becker's Payer, WARN filings, Modern Healthcare, Healthcare Dive), attributed in-line as public reporting. The humility clause appears once, on Huber, the contract-owning seat. No Intradiem ROI, savings, idle-percentage, or outcome number appears anywhere; Humana is the only customer proof permitted, replies only, as Humana's own public telling. The UHC \$190M figure and the CMS June 2026 recalculation are excluded by design.

Execution notes (one-off, stays out of the Clay workbooks).

The committee, copy, and data live in this document; nothing lands in a Clay motion workbook. Emails, all resolved through Clay Jul 16: 6 of 8 found by the work-email waterfall (Iambert, wynant, furman, karrobi, uhm, mckinney @blueshieldca.com); Mathews and Huber missed the waterfall and were resolved by a Clay deep-find datapoint (carolyn.mathews@ returned directly, george.huber@ as the stated pattern fallback after no public listing). ZeroBounce sweep RUN Jul 16 (scratch table OneOff_CardinalHealth_ZB_Jul15, 0.1 credits/row): 8/8 valid. Six addresses show catch_all sub-status (catch-all domain, incl. Mathews and Huber); Karrobi and Uhm verified clean. Pre-send email gate is CLEARED; remaining gates are rep review and the Salesforce ownership check. Sends run through the rep's one-off surface, Draft until rep review; nothing fires without a human approving each message. Before launch, Dallas flags in Slack for the standard checks: Salesforce ownership and open-opportunity check on Blue Shield of California (AE lane: Jean-Anne McMahon), plus mailbox warm-up status. A second ring (Stars program leads under Furman/Huber, member-services directors under Lambert/Mathews, WFM leadership) is a fresh one-off pull on Nathan's go.

Credits this one-off: contact search free; 8-contact email waterfall via Clay MCP, estimated ~12-20 credits, plus 2 deep-find Custom datapoint runs (meter-confirm all on the Usage page next Clay session); ZeroBounce actual 0.8 (8 rows x 0.1, Jul 16). Logged in Clay_Credit_Ledger.md.